Clinical Notes in Practice: Admission, Progress, Nursing and Handover

The entries that make up most of a file, and what separates a useful note from a filled page.

Module	04 of 12
Study time	105 minutes
Audience	Doctors, nurses, allied health, clinical documentation improvement staff
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition AAC chapter on assessment, reassessment and continuity; COP chapter; IMS chapter

Admission notes, progress notes, nursing notes and handover records make up the bulk of every file and almost all of its evidential weight. They are also where documentation most often degrades into ritual — pages that are filled in without saying anything. This module sets out what each of these entries is for, what must be in it, how often it must be written, and how to write one that a stranger can act on.

LEARNING OUTCOMES — by the end of this module you will be able to:

- Document an initial medical and nursing assessment to the required content and within the required time
- Write a progress note that records findings, reasoning and plan rather than a conclusion alone
- Set and document reassessment intervals appropriate to the patient's acuity
- Complete the nursing record set, including risk screens, care plans and flow sheets
- Document escalation of a deteriorating patient so that the sequence is defensible
- Record a structured handover and a transfer between units
- Identify and avoid the common failure patterns in each of these documents

4.1 The first twenty-four hours



Figure 4.1 — What must be documented in the first twenty-four hours, and by when.

The admission period generates the documents that every later reader depends on. If the initial assessment is thin, every subsequent note is built on an undocumented foundation, and the hospital cannot later show what the patient's baseline was — which is exactly what is argued about when a patient deteriorates.

The initial medical assessment

NABH requires a documented initial assessment within a timeframe defined by the organisation, and for inpatients this is conventionally within twenty-four hours of admission or earlier where the patient's condition demands. The content requirement is more demanding than the timing requirement, and it is the content that is usually deficient.

Element	What a complete entry contains	Common deficiency
Presenting complaint and history of present illness	Onset, duration, progression, associated features, what has already been tried, and in the patient's own words where that matters	A single phrase such as 'fever 3 days' with no character, no progression and no negatives
Past medical and surgical history	Conditions, prior procedures with dates, prior admissions, and relevant family history	Copied from an old discharge summary without checking currency
Medication history	Every drug the patient was taking, with dose and frequency, including over-the-counter, traditional and herbal preparations and insulin	'On regular medication' with no list; this is also the root of failed reconciliation
Allergy status	Drug, food and other allergies with the reaction described, or an explicit 'no known allergies'	Field left blank, which is indistinguishable from not asked
Examination findings	General condition, vital signs with values, systemic examination relevant to the presentation, and significant negatives	'P/A soft, CVS S1S2, RS clear' recorded identically for every patient on the round
Provisional and differential diagnosis	A working diagnosis and what else is being considered and why	A symptom recorded as a diagnosis: 'fever for evaluation' left unchanged until discharge
Initial plan	Investigations with the question each one answers, treatment started, monitoring required, and the parameters that would trigger escalation	A list of investigations with no clinical reasoning and no monitoring plan

> **CAUTION**
> The twenty-four hour rule fails most often for the patient admitted late at night, the patient transferred from casualty to a ward under a consultant who has not yet seen them, and the patient admitted over a weekend. Those are the three files an assessor will look for, because they are the three where the assessment is most likely to be missing.

4.2 The initial nursing assessment and risk screens

The nursing assessment is not a shorter version of the medical one. It records functional status, nutrition, skin condition, continence, mobility, psychosocial circumstances, home support and the patient's own understanding of their admission — none of which appears in the medical note, and all of which determine the care plan.

Risk screens must be scored, not described. A falls screen that says 'at risk' without a score cannot be compared across shifts, cannot be re-scored after a change, and cannot evidence that the hospital's own protocol was followed. The score must also trigger something: a screen that produces a number and no intervention is a wasted form.

Screen	What must be documented	What the score must trigger
Falls risk	Tool used, score, date, time, assessor, and reassessment after any fall or change in condition	Falls precautions documented in the care plan; patient and family informed; environment reviewed

Screen	What must be documented	What the score must trigger
Pressure injury risk	Tool used, score, skin inspection findings including existing lesions with site and stage	Repositioning schedule, support surface, nutrition referral, and documented skin reinspection
Nutritional screening	Tool used, score, weight, recent weight loss, intake	Dietitian referral at the defined threshold, and a documented nutrition plan
Pain	Tool appropriate to the patient, score at rest and on movement, site and character	Analgesia given and reassessment within the defined interval, with the post-intervention score recorded
Venous thromboembolism, where applicable	Risk factors, bleeding risk, and the decision taken	Prophylaxis prescribed or an explicit documented reason why not

4.3 The progress note

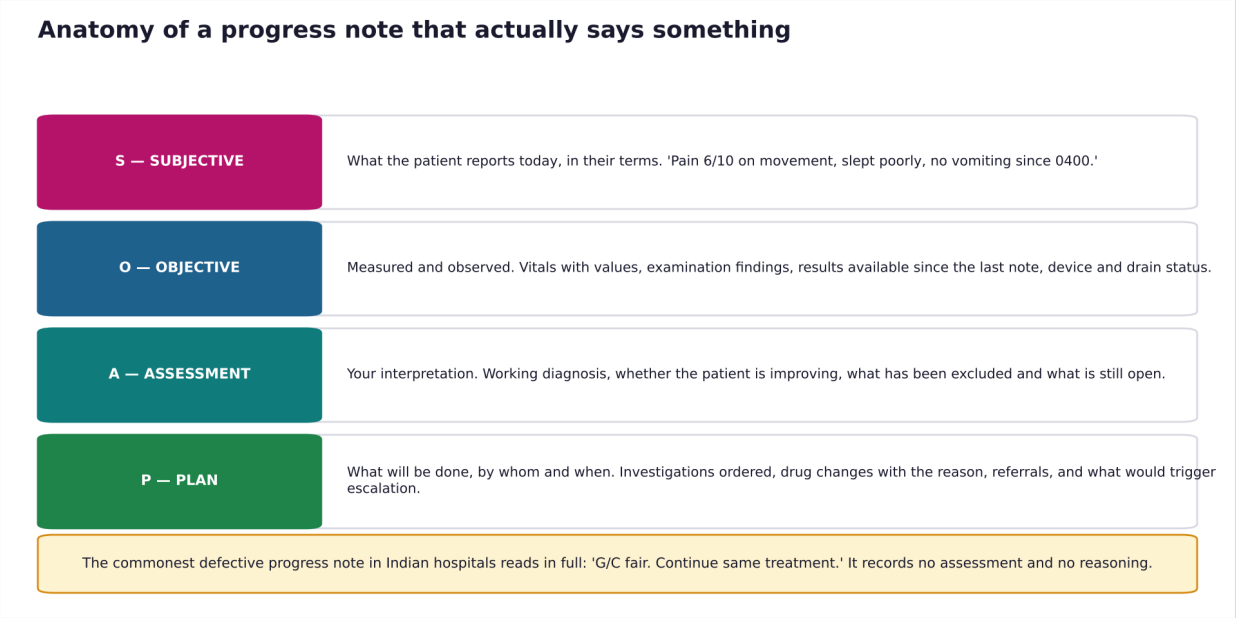


Figure 4.2 — The anatomy of a progress note that actually says something.

A progress note has one job: to let the next clinician pick up the patient without starting again. That requires the note to record not only what was observed but what was concluded and what is intended. The most common defective note in Indian hospitals reads “G/C fair. Continue same treatment.” It contains no assessment, no reasoning, no plan and no trigger, and it is written identically for a patient improving and a patient about to deteriorate.

Two versions of the same patient

Thin note	Note that carries the clinical reasoning
Day 3 post-op. G/C fair. Afebrile. Wound healthy. Continue same. — signature illegible	**14/03, 09:20.** Day 3 after open appendicectomy. Patient reports pain 3/10 on movement, tolerated soft diet overnight, passed flatus at 06:00. Temperature 37.1, pulse 84, BP 118/74, respiratory rate 16, saturation 98 per cent on air. Abdomen soft, bowel sounds present, wound dry with no surrounding erythema, no discharge. Impression: progressing normally; no evidence of surgical site infection. Plan: continue oral analgesia, stop intravenous fluids, full diet today, mobilise, remove cannula. Likely discharge tomorrow if apyrexial and taking diet. Escalate if temperature above 38, increasing pain or wound discharge. — Dr A Kumar, Surgical Registrar, Reg. No. 12345

The second note is longer, but it takes about ninety seconds to write and it answers every question any later reader will have: the nurse deciding whether to call overnight, the consultant on the weekend round, the coder looking for the final diagnosis, the claims assessor asking why the patient stayed four days, and the lawyer asking whether the deterioration was foreseeable.

> **KEY POINT**
> The single most valuable sentence in a progress note is the escalation trigger — the statement of what would make you want to be called. It converts a note into an instruction and it is the sentence most often missing from files in cases about delayed response.

4.4 Frequency and reassessment

Reassessment intervals are set by the patient's condition and by the hospital's policy, and both the interval and the actual reassessment must be documented. A patient whose chart shows four-hourly observations ordered and six-hourly observations recorded has an unexplained gap that will be read as a failure of monitoring.

Patient category	Typical documented reassessment	What the record must show
Stable ward patient	Medical review daily; nursing assessment each shift; vitals as ordered	A dated, timed entry each day and each shift, with no unexplained gaps
Patient on a deteriorating trajectory or with an abnormal early warning score	Interval shortened per protocol, with escalation documented	The score, the action taken, who was informed, at what time, and what they advised
Critical care	Hourly or continuous charting, with a documented review at defined intervals	Flow sheets totalled per shift, ventilator and infusion settings, and the reasoning behind each change
Post-operative	Recovery observations per protocol, then ward observations at defined decreasing frequency	Time of return to ward, observation set on arrival, pain scores, and the first post-operative review
Patient receiving a high-risk medication or transfusion	Observations at the intervals specified in the relevant policy	Pre-administration baseline, the specified intervals actually recorded, and any reaction or an explicit record of none
Palliative or end-of-life care	As agreed in the documented plan of care, which may reduce observations deliberately	The agreed plan, the discussion with the patient or family, and the clinical rationale for any reduction

4.5 The nursing record set

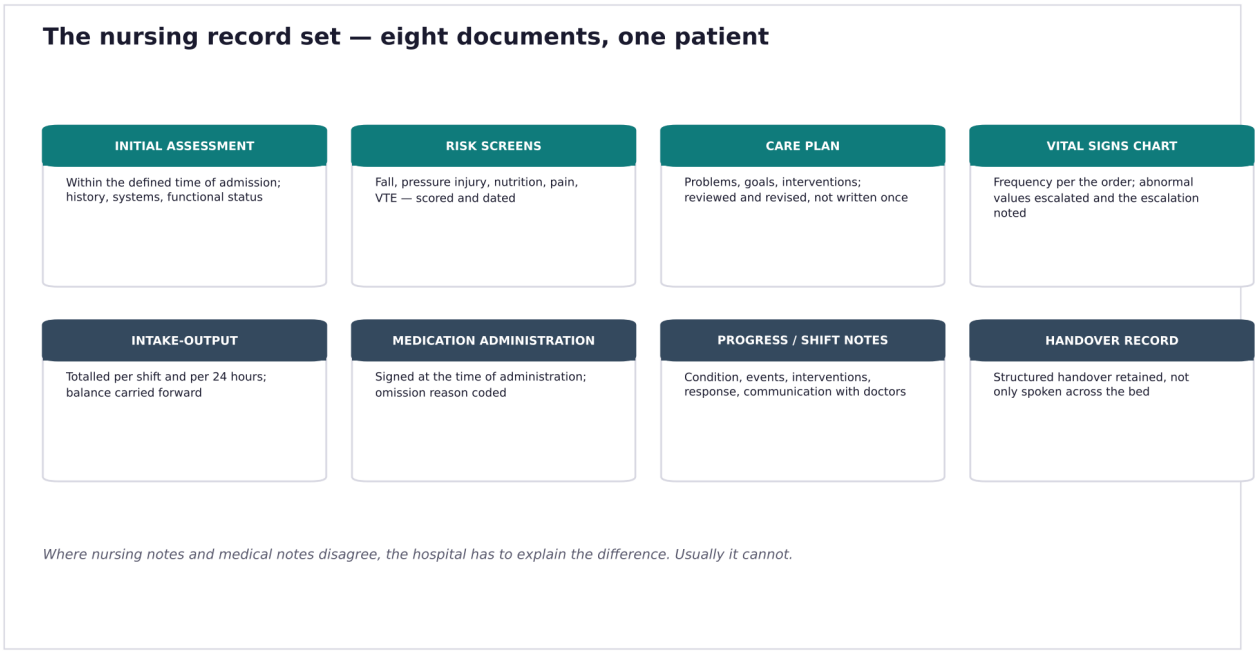


Figure 4.3 — The eight documents that make up the nursing record.

Nursing records decide a large proportion of disputed cases, because nurses are present continuously and their charts carry the timeline. Three parts of the set deserve particular attention.

The care plan

A care plan is a living document: problems identified, goals set, interventions planned, and then reviewed and revised as the patient changes. A care plan written on admission and never touched again evidences nothing except that a form was completed. The review must be dated and signed, and where a goal is not met the record should say what was changed.

Flow sheets

Vital signs charts and intake-output charts are the most quoted documents in litigation, because they show trends. Three disciplines matter: values must be plotted or written at the time; totals must be calculated at the end of each shift and each twenty-four hours, with the balance carried forward; and abnormal values must be linked to an action. A chart that records a rising pulse over six hours with no corresponding note is the clearest possible evidence of a missed deterioration.

Shift notes

Each shift should produce a narrative note covering the patient’s condition, significant events, interventions and responses, communication with doctors and with the family, and the state in which the patient was handed over. “Routine care given, patient comfortable” for an eight-hour shift on a sick patient is not a record.

4.6 Documenting escalation

Escalation is the single highest-risk documentation sequence in a hospital, because the allegation in these cases is always about delay. The record must be able to answer four questions without ambiguity: when did the abnormality appear, who was informed and when, what did they say, and what happened next.

1. Record the abnormal observation with its exact time and value, and the early warning score if your hospital uses one.
2. Record the action taken immediately — repeat observation, oxygen, position, fluids — with times.
3. Record who was contacted, by what means, at what time, and record that you described the specific findings.
4. Record what the clinician advised, verbatim where it is an instruction, and whether they said they would attend.

5. Record the time the clinician actually attended. A gap between the call and the attendance is a fact; leaving it undocumented does not make it disappear.
6. If there is no response, record the escalation to the next level and the time, and keep escalating until there is a response.
7. Record the outcome and the subsequent observations.

> **CAUTION**
> Never write “informed doctor” without the name and the time. In proceedings, an unnamed and untimed communication is treated as not having happened, and the nurse who made the call is left unable to prove she made it.

4.7 Handover and transfer

ISBAR — structuring the handover so it can be written down

	Stands for	What goes in the written handover
I	Identify	Who you are, who you are speaking to, and the patient: name, MRN, bed, consultant
S	Situation	Why you are handing over now: 'post-op day 1, now spiking to 39°C'
B	Background	Admission diagnosis, relevant history, allergies, procedures done, current drugs
A	Assessment	Your reading: vitals, scores, what you think is happening, what you have ruled out
R	Recommendation	What you want done, by when, and what to escalate on — with read-back confirmed

A handover that is only spoken leaves no evidence that the receiving clinician was told about the deterioration.

Figure 4.4 — ISBAR, structuring a handover so that it can be written down.

Handover is a clinical act and a documented one. A spoken handover leaves no evidence that the receiving clinician was told about a deterioration, an outstanding result or a pending decision. The written handover record should be retained for the admission, not discarded at the end of the shift.

Transfer between units

A transfer — ward to ICU, ward to theatre, hospital to hospital — is where information is most often lost, because each side assumes the other documented it. The transfer record must state the reason for transfer, the patient’s condition at the time, the treatment given and due, outstanding investigations, lines, drains and devices in situ, allergies, the receiving unit and the person accepting, and it must be signed by both sides.

Transfer type	What is most often lost	What the record must capture
Ward to operating theatre	Allergy status, consent verification, fasting status, implants and devices	The pre-operative checklist, completed and signed, accompanying the patient
Theatre to recovery to ward	Analgesia already given, blood loss, drains, and the next dose due	The anaesthesia record, the recovery record with discharge criteria met, and a verbal plus written handover
Ward to intensive care	The trajectory that led to transfer, and what has already been tried	The deterioration sequence with times, interventions and responses, not merely the current state
Intensive care to ward	The reason for every ongoing therapy and the plan for stopping it	A step-down summary with current therapy, rationale, monitoring required and escalation triggers

Transfer type	What is most often lost	What the record must capture
Hospital to hospital	Investigation results, imaging, and the clinical reasoning	Transfer summary, copies of results, imaging on media or shared electronically, named accepting doctor, mode of transport and condition at departure

4.8 Documenting what the patient was told

A substantial proportion of complaints are not about clinical decisions but about communication: that nobody explained, that the family was not warned, that the patient was not told the result. The only defence is a contemporaneous note of the conversation. It need not be long, but it should record who was present, what was explained, what questions were asked, and what was agreed.

This matters most for bad news, for a change in prognosis, for a decision to limit treatment, for a complication that has occurred, and for a refusal of recommended treatment. Each of these should produce a dated, timed note naming the participants.

> **KEY POINT**
> Where a complication has occurred, document the disclosure conversation factually and promptly: what happened, what is being done about it, and what the patient was told. A documented, early and honest disclosure is both the right thing clinically and the single most protective entry the hospital can have.

4.9 Pain documentation

Pain is assessed, scored, treated and reassessed, and each of those four steps must appear in the record. The reassessment is the step most often missing, and it is the one that shows whether the treatment worked.

Step	What to record
Assessment	Site, character, radiation, severity using the hospital's tool, what makes it better or worse, and its effect on function such as sleep, breathing and mobility
Tool used	The tool appropriate to the patient: a numerical scale for adults who can report; a faces scale for children; an observational tool for patients who cannot self-report
Treatment	What was given, the dose, the route and the time; and non-pharmacological measures taken
Reassessment	The score after the intervention, within the interval the policy specifies, with the time; and the action taken if it has not improved

For a patient who cannot self-report — sedated, ventilated, cognitively impaired, very young — the record must show which observational tool was used and what the behaviours observed were. 'Patient comfortable' is not an assessment.

4.10 Restraint documentation

Physical or chemical restraint is a high-risk intervention with a documentation standard to match. The record must justify it, define it, monitor it and end it.

- The clinical indication, and what less restrictive alternatives were tried and failed
- The authorisation: who ordered it, their designation, the date and time; a restraint order is time-limited and must be renewed, not left open
- The type of restraint and the specific limbs or method used
- The consent or, where the patient lacks capacity, the basis on which it proceeds, and that the family was informed
- Monitoring at the frequency the policy specifies: circulation, skin integrity, hydration, toileting, position change, and the patient's mental state

- Review at the defined interval with an explicit decision to continue or discontinue
- The time restraint was removed, and the patient's condition afterwards
- An incident or event record where the hospital policy requires it

> **CAUTION**
> An open-ended restraint order with no review interval and no monitoring record is one of the most serious documentation findings an assessor can make, and in the event of a harm it is indefensible.

4.11 Vulnerable patients

NABH requires that vulnerable patients be identified and that their additional needs be addressed and documented. The groups include children, the elderly, patients with disability, patients who are unconscious or sedated, patients with mental illness, and patients who are victims of violence or abuse.

Group	What the record must additionally show
Children	Guardian identified and relationship verified; weight-based dosing; safeguarding concerns recorded and escalated through the designated route
Elderly and frail patients	Falls and pressure risk assessed and acted on; cognitive status; capacity where decisions are required; discharge planning including home support
Patients with disability	Communication needs and how they were met; mobility and assistance required; consent obtained in an accessible way
Unconscious or sedated patients	Who is making decisions and on what basis; pressure, eye and mouth care; positioning; and the plan for reassessment of capacity
Patients with mental illness	Capacity, advance directive, nominated representative, statutory admission category, and restraint documentation where applicable
Victims of violence or abuse	Objective injury documentation, medico-legal declaration, protection measures taken, and referral to the designated officer; the identity of the patient protected

4.12 Allied health documentation

Physiotherapy, dietetics, speech and language therapy, occupational therapy, medical social work and clinical psychology all contribute to the record, and their entries are part of it. Each should record an assessment, a plan with goals, the intervention delivered, the patient's response, and a discharge or handover statement.

Two failures recur. The first is that allied health notes sit in a separate file that the treating team never reads, so a dietitian's finding of severe malnutrition never reaches the discharge summary. The second is that the recommendations are recorded without any record of whether they were acted on. Both are fixed by filing allied health entries within the main record and by requiring the treating team to respond to recommendations in the progress notes.

4.13 Family communication

Families are told a great deal in Indian hospitals and almost none of it is recorded. A short note — who was present, what was explained, what was asked, what was agreed — takes a minute and answers the most common complaint a hospital receives.

- Record the name and relationship of the family member spoken to, not 'attender informed'
- Record what was explained about the diagnosis, the prognosis and the plan
- Record explicitly where a complication or an adverse event has been disclosed
- Record the patient's own wishes about what may be shared with family, particularly where they differ

- Record any refusal by the family on the patient's behalf, and whether the patient has capacity to decide for themselves
- Record the time; in a deteriorating patient, the sequence of what the family was told and when is often the whole of the dispute

4.14 Putting this module to work

1. Read the last five progress notes in a current file. Count how many state an assessment and a plan, as opposed to a conclusion alone.
2. Take one vital signs chart and check whether intake and output are totalled for each shift and carried forward.
3. Find the last entry on your ward that says 'informed doctor' and check whether it names the doctor and gives the time.
4. Look at one admission from the last weekend and check whether the initial medical assessment is present and within twenty-four hours.
5. Check whether your ward's handover record is retained or discarded at the end of the shift.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A patient's pulse rises from 88 to 124 over six hours, recorded faithfully on the chart. There is no clinical note in that period. What will be concluded, and what should have been recorded?**

Answer: That the deterioration was documented and not acted upon, which is worse for the hospital than not having charted it at all, because the trend is now proved. Each abnormal value should have prompted a note: the observation, the early warning score, the immediate nursing action, who was informed and when, what they advised, when they attended, and the subsequent observations. The gap between recording and responding is the defect, and it is a clinical defect before it is a documentation one.

Q2. **An intern writes 'G/C fair, continue same' for a post-operative patient on day two. The patient develops an anastomotic leak on day three. How does this note affect the hospital's position?**

Answer: It leaves the hospital unable to show what was examined on day two. There is no record of abdominal findings, drain output, temperature trend, bowel function or pain, so it cannot be demonstrated that the leak was not detectable at that point. The note neither helps nor is neutral; it creates a gap on the critical day. A note recording the examination, the drain output, the pain trajectory and an explicit escalation trigger would have shown reasonable care even if the leak was later confirmed.

Q3. **A nurse completes a falls risk screen as 'high risk' and nothing else changes in the record. The patient falls that night. What is the documentation failure?**

Answer: The screen produced a score and triggered nothing. The record should show the falls precautions that were put in place, the patient and family being informed, the environment being reviewed, the care plan being updated to reflect the risk, and reassessment at the defined interval. A risk identified and not acted upon is evidence that the hospital knew of the risk and did nothing, which is a worse position than not having screened.

Q4. **At handover a nurse tells the incoming shift that a potassium result is pending and abnormal results should be escalated. She writes nothing. The result returns high overnight and is not acted on. What follows?**

Answer: The hospital cannot show that the incoming shift was informed, so the failure is attributed to the handover as well as to the overnight response. A written handover record naming the outstanding investigation, and a note in the clinical record that a critical result was awaited with the escalation instruction, would have fixed responsibility and prompted action. Spoken handover without a record is the commonest single point of information loss in a hospital.

Q5. **A consultant explains to a family that the patient's prognosis is poor and that escalation to intensive care would not be beneficial. Nothing is recorded. The patient dies and the family alleges they were never told. What is the hospital's position?**

Answer: Indefensible on the communication point, regardless of whether the clinical decision was correct. The conversation should have generated a dated, timed note naming who was present, what was explained about the condition and prognosis, what options were discussed including intensive care, what was decided and why, and what the family said. Decisions to limit treatment require documentation at the same standard as the decision to give it.

Q6. **A patient is transferred from the ward to intensive care. The transfer note records only 'Shifted to ICU for further management'. What is missing and why does it matter clinically?**

Answer: Everything the intensive care team needs: the trajectory and times of the deterioration, the observations and scores, what interventions were already tried and the response, current drugs including the last doses given, allergies, lines and devices in situ, outstanding investigations, and the name of the clinician accepting. Without the trajectory the receiving team restarts the assessment from the current state and may repeat failed interventions or miss the pattern that led to the transfer.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.